

Hospital Intake Workflow Optimization

Project Charter & Management Plan | Simulated Portfolio Case Study

Data disclosure: All data is simulated. This project contains no PHI or real patient information.

1. Executive Summary

This project coordinates a simulated hospital intake workflow optimization across registration, insurance verification, digital intake, and clinical handoff. The objective is to reduce processing delays, lower rework, improve data quality, and standardize intake across five departments.

2. Business Need

The current-state workflow includes duplicated intake information, late insurance verification, inconsistent handoffs, and exception handling that occurs too late. These conditions create avoidable wait time and rework.

3. Objectives

Reduce average registration time by at least 40%; reduce average wait time by at least 35%; increase digital form adoption to at least 90%; reduce simulated data-entry errors below 3%; improve patient satisfaction to at least 4.3/5; complete all milestones; close all critical risks/issues; and remain within the \$105,000 baseline.

4. Scope

In scope: As-Is mapping, KPI baseline, To-Be design, digital intake configuration, pre-arrival eligibility workflow, staff training, pilot, QA, phased rollout, stabilization, reporting, and closeout. Out of scope: EHR replacement, clinical treatment redesign, claims adjudication, staffing model redesign, and use of real patient records.

5. Success Measures

Metric	Target	Simulated Result	Status
Registration time reduction	>=40%	48.6%	Met
Wait-time reduction	>=35%	44.5%	Met
Digital form adoption	>=90%	94%	Met
Data error rate	<3%	1.8%	Met
Patient satisfaction	>=4.3/5	4.6/5	Met
Milestone completion	100%	100%	Met
Budget	<=\$105,000	\$103,150	Met

6. Workstreams & Deliverables

Workstream	Deliverables	Primary Owner
Governance	Charter, status cadence, decision log, closeout	Project Coordinator
Process Mapping	As-Is / To-Be maps, bottleneck findings	Business Analyst / Process Lead
Digital Intake	Form configuration, validation, mapping	IT Applications Lead
Eligibility	Pre-arrival insurance work queue	Revenue Cycle Lead
Training	Standard work, job aids, staff sessions	Training Lead
Pilot & QA	Pilot, defect log, readiness criteria	Project Coordinator / IT Apps
Rollout	Phased deployment and stabilization	Project Coordinator
Analytics	Operational KPI dashboard and reporting	Data Analyst / Project Coordinator

7. Governance & RACI

Weekly working-team meetings reviewed tasks, dependencies, RAID, defects, and changes. Pilot-period huddles were held daily for operational exceptions. Executive updates were issued biweekly.

Activity	Sponsor	Project Coord.	Registration	Revenue Cycle	Nursing	IT Apps
Charter	A	R	C	C	C	I
As-Is mapping	I	C	R	R	R	C
To-Be design	I	R	C	C	C	C
Digital configuration	I	C	C	I	I	A/R
Eligibility redesign	I	C	C	A/R	I	C
Pilot	I	A/R	R	R	R	R
Full rollout	I	A/R	R	R	R	R
KPI reporting	I	A/R	C	C	C	C

8. Schedule & Milestones

Milestone	Baseline	Actual	Variance
Charter Approved	Feb 4, 2026	Feb 4, 2026	0 days
As-Is Mapping Complete	Feb 9, 2026	Feb 9, 2026	0 days
To-Be Design Approved	Feb 17, 2026	Feb 18, 2026	+1 day
Pilot Ready	Mar 7, 2026	Mar 6, 2026	-1 day
Pilot Exit	Mar 18, 2026	Mar 18, 2026	0 days
Full Rollout Complete	Mar 29, 2026	Mar 29, 2026	0 days
Executive Closeout	Apr 13, 2026	Apr 13, 2026	0 days

9. Risk & Issue Management

ID	Item	Score	Mitigation / Resolution	Status
R-01	Digital adoption risk in Emergency Intake	12	Assisted digital station + staff script	Closed
R-02	Eligibility backlog may shift delay upstream	15	Queue threshold + backup coverage	Closed
R-03	Department practices may diverge	12	Standard work + audit checklist	Closed
I-01	Duplicate guarantor mapping defect	20	Corrected mapping + retest	Closed
D-01	Clinical leadership sign-off	20	Readiness review with pilot evidence	Complete

10. Budget Management

Category	Baseline	Actual	Variance
Workflow Analysis & Design	\$18,000	\$17,400	+\$600
Digital Intake Configuration	\$26,000	\$27,150	-\$1,150
Eligibility Workflow Setup	\$15,000	\$14,400	+\$600
Training & Change Management	\$16,000	\$15,350	+\$650
Pilot & QA	\$12,000	\$11,800	+\$200
Rollout & Stabilization	\$18,000	\$17,050	+\$950
TOTAL	\$105,000	\$103,150	+\$1,850 favorable

11. Quality & Acceptance

Acceptance criteria included validated digital-form rules, eligibility queue readiness, complete staff training, resolved critical pilot defects, standardized handoff checklist, KPI trend improvement, and executive acceptance of closeout.

12. Privacy & Data Handling

The portfolio artifact uses only simulated operational metrics. No patient names, dates of birth, identifiers, diagnoses, insurance numbers, encounter records, or other PHI are included.

13. Lessons Learned

- Move insurance verification upstream only when queue ownership and exception thresholds are clear.
- Real-time form validation prevents downstream rework more effectively than end-of-process review.
- Pilot exit criteria should include both defect severity and operational adoption metrics.
- Department-specific adoption support may be necessary even when the standardized workflow is the same.
- One formula-driven dashboard reduces inconsistency between operations and executive reporting.